

DEMO DAY · 25 MINUTES

Tie Me Ghana Speaker Notes

What to say on each screen. The opening words. And how to answer the question, "Where can we use this?"

How to use these notes

The green boxes are the words to say out loud. You can read them as they are. Short lines are better than long ones. Stop after each line. Let it land.

The grey line at the end of each screen tells you what to click next.

- **Speak slowly.** Slow sounds sure. Fast sounds nervous.
- **Look at the judges, not the screen.** You know the app. They do not.
- **Say what a thing is for before you show it.** One line is enough.
- **Do not say sorry** if something is slow. Just keep talking.

Before you start

- Open the site on both devices and let the sign videos play once. They are then saved on the device and load fast.
- Put the phone and the laptop on the same Wi-Fi.
- Turn the sound up. One part of the demo speaks out loud.
- Have the demo already open at the first screen before you talk.
- Make sure the newest build is live on the web site before the day.

The opening (about 90 seconds)

START HERE. DO NOT TOUCH THE LAPTOP YET.

Good morning. We are team **[team name]**. Our project is called **Tie Me Ghana**.

Think of a deaf patient who walks into a hospital in Ghana today.

The doctor speaks. The patient cannot hear. The doctor writes it down. Many deaf people in Ghana did not finish school, so they cannot read it.

So the hospital looks for a sign language helper. There are very few. Often there is none. So the visit goes on with hand waving and guessing.

That is the problem. A patient who cannot tell you where the pain is. A doctor who cannot be sure the patient understood the medicine.

Tie Me Ghana lets that doctor and that patient talk to each other. Today. With the phone already in their hand.

Then say: "Let me show you." Turn to the screen.

Three lines that make it feel real

Use one of these in the opening if you want more weight. Only say a number you have checked yourself.

- “Ghana has about **[number]** deaf people, and only **[number]** trained sign language helpers.”
- “A hospital cannot hire a helper for every ward. But every ward already has a phone.”
- “This is not a new gadget. It is the gap between a doctor and a patient, closed with what the hospital already owns.”

The demo, screen by screen (about 10 minutes)

1 “Does this patient have their own phone?”

40 sec

WHAT THEY SEE

The first screen of a visit. Two big buttons. Nothing else.

WHAT YOU SAY

This is the first question the app asks the nurse. Not the patient’s name. Not a form. Just this.

If the patient has a phone, we use two screens. The doctor keeps his own, and the patient holds hers.

If the patient has no phone, we use one screen and turn it between them. Both ways work. We did not want the poorer patient to get the worse app.

Do: tap *Yes, pair it*.

2

The pairing code

50 sec

WHAT THEY SEE

Six letters on the doctor's screen. You type them into the phone.

WHAT YOU SAY

Six letters. The patient types them once, and the two devices are joined.

Now the important part. Once they are joined, the two devices talk **straight to each other**. Nothing the doctor and the patient say goes through our server.

Our server only helped them find each other. Like giving someone a phone number. After the call starts, it hangs up.

So the answers, the body map, the medicines — none of it is stored by us. That was a health data choice, not a clever one.

Do: type the code on the phone. Wait for "Connected".

3

"Can this patient read and write?"

50 sec

WHAT THEY SEE

The same question on both screens at the same time. With a sign video.

WHAT YOU SAY

Here is the one question that changes everything after it.

Notice it is on the patient's phone too, in sign language. Because this is a question **about** the patient. She should be the one who answers it, not the nurse guessing.

If she reads, we give her words. If she does not read, we give her pictures, a body map, and sign videos. Same app. Two roads.

Do: tap *No* on the phone, to show the harder road first. That is the one judges care about.

4 The doctor asks. The patient answers.

2 min

WHAT THEY SEE

The doctor picks a question. A sign video plays on the patient's phone. The patient taps *Yes* or *No* — and those buttons have their own sign clips on them.

WHAT YOU SAY

The doctor asks: "Do you have pain?" He taps it. He does not type it.

On the patient's phone, a real person signs the question. That is Ghanaian Sign Language. Not American. We filmed it here.

Look at the two buttons. Even *Yes* and *No* are signed. A patient who reads nothing at all can still answer this screen.

She taps. The answer appears on the doctor's side as plain English.

Now the body map. She points at where it hurts, on a drawing of a body. Then she picks how bad it is, on a scale with faces.

In one minute, the doctor knows: there is pain, it is here, and it is this bad. Without one word of writing.

Do: run one Yes/No question, then the body map, then the pain scale. Do not run more than that. You will run out of time.

5 The other road: the patient who can read

70 sec

WHAT THEY SEE

The doctor types a free sentence. It is shown in Twi, and played as signs. The patient types back, and the doctor's device speaks it out loud.

WHAT YOU SAY

This road is for a patient who reads. Here the doctor is not stuck with set questions. He types what he wants.

We show it two ways. In Twi as text. And as sign video, for the words we have filmed.

When the patient types back, listen — the doctor's device **says it out loud**. So the doctor can keep his eyes on the patient, not on the screen.

Do: turn the sound up before this one. Type a short sentence you have tested.

6

When the app says no

60 sec

WHAT THEY SEE

The doctor types a sentence with a word that has no sign clip. The app **refuses to play it** and says why.

WHAT YOU SAY

This is the part we are most proud of, and it is the part that does nothing.

This sentence has a word we have not filmed. A drug name, say.

A normal app would drop that word and play the rest. The patient would see a sentence that looks fine and means something else. In a hospital, that is how you get the wrong dose.

So we refuse. The app tells the doctor which word is missing and asks him to say it another way.

We would rather show nothing than show a half sentence.

Do: use a sentence you have already tested, so you know it will be refused.

7

Emergency

60 sec

WHAT THEY SEE

The doctor taps *Emergency*. Both screens change at the same moment. Big red buttons: cannot breathe, chest pain, bleeding, pregnant.

WHAT YOU SAY

Now think about a deaf patient who arrives at night, alone, and cannot breathe.

One tap, and both screens change together. The patient does not have to be told. Her phone is already showing it.

Four buttons. Big. Red. She taps one, and the nurse has the emergency in English right away.

This screen is open before the visit even starts. You should not have to finish a form to say you cannot breathe.

Do: tap one red button on the phone so the judges see it land on the doctor's side.

WHAT THEY SEE

The doctor builds a short medicine list. On the patient's phone: a QR code, sign videos for each medicine, and a *Save* button.

WHAT YOU SAY

This is where most visits go wrong, for everybody, deaf or not. The patient leaves, and by the gate she has forgotten half of it.

The doctor picks the medicine, how much, and how often. Three taps.

Look at her phone. She gets it as sign video. One clip for each instruction. She can play it again as many times as she likes.

And she taps *Save*. Now it is on her phone. She can watch it again at home, tonight, when the doctor is not there.

The QR code is for the pharmacy, so nobody has to read the doctor's handwriting.

Do: tap *Save* on the phone and show that the file is there. This is the strongest moment of the demo.

WHAT THEY SEE

Go back and start a visit with *No, share this device*. One screen, split by a line. "Doctor" on one side, "Patient" on the other.

WHAT YOU SAY

Most patients will not have a smart phone. So here is the same visit on one screen.

See the line down the middle. This half is the doctor's. That half is the patient's. Turn the screen, and each person knows which part is theirs.

A small thing. But we watched people get confused without it.

Do: keep this short. Show it and move on.

10 No app store. No download.

40 sec

WHAT THEY SEE

The browser's *Add to Home Screen*. The app icon on the phone.

WHAT YOU SAY

Last thing. There is nothing to install.

It is a web site. It opens in any browser, on any phone, on any old laptop the ward already has.

But add it to the home screen, and it becomes an app with an icon. The sign videos are saved on the phone after the first time, so it stays fast even on a weak network.

A hospital does not need new machines to use this. It needs the link.

The closing (30 seconds)

So that is Tie Me Ghana.

A deaf patient can say where the pain is. A doctor can be sure she understood the medicine. And nobody had to find a sign language helper.

It runs today, on the phones people already carry, in Ghanaian Sign Language we filmed here.

It is a pilot. It is not approved for clinical use yet, and we will say so on every screen until it is.

Thank you. We are happy to take your questions.

"Where can we access it?"

Expect this one. Answer it fast and clearly. Say the link out loud, letter by letter if you must, and show it on the screen.

Say this

It is live right now. You can open it on your own phone in this room.

The doctor's side is at **tiemeghana.netlify.app**.

The patient joins at **tiemeghana.netlify.app slash join**.

There is nothing to download. No app store. It is a web site, so it opens in Chrome, Safari, any browser, on a phone, a tablet or a laptop.

If you want it to look like an app, open it once and tap "Add to Home Screen". Then it has an icon like any other app.

WHO	WHERE THEY GO
Doctor or nurse	https://tiemeghana.netlify.app
Patient	https://tiemeghana.netlify.app/join
Judge who wants to try it now	Open the first link. Choose “No, share this device”. You do not need a second phone.

If they ask a harder version of the same question

Q. Is this a real deployment, or a demo you run on your laptop?

“It is a real deployment. The site is hosted online, the database is online, and the sign videos are served from cloud storage. What you are watching is not running on this laptop.”

Q. Can we see the code?

“Yes. The code is in our repository and we are happy to give the judges access. Tell us the email to add.” Do not promise a public link on stage if the repository is private.

Q. How do we reach your team after today?

“I am **[name]**, and the best way is **[email or phone]**. We will answer the same day.” Agree on one person and one address before you go on stage. Do not work it out in front of the judges.

Q. Is it in an app store?

“No, and that is on purpose. An app store means a download, a data bundle, and a phone with enough space. A link works on every phone, today. We can still package it later if a hospital asks.”

Other questions they will ask

Keep every answer to three lines. If you do not know, say “We have not solved that yet” and move on. Judges trust that more than a guess.

Q. How many signs do you have? Is that enough for a real hospital?

“We have 64 reviewed clips today, filmed with deaf signers. That is enough for the questions in this demo, and not enough for a whole hospital. That is why the app refuses a sentence it cannot sign properly instead of guessing. Adding a sign is filming it and having it reviewed — not changing the code.”

Q. Why not use a computer to make the signs?

“Sign language is faces and body, not only hands. A machine avatar gets it wrong, and in a hospital wrong is dangerous. So every clip is a real deaf signer, and a reviewer has to approve it before a patient can see it.”

Q. What about patient privacy? This is health data.

“Once the two devices are joined, they talk straight to each other. What the doctor asks and what the patient answers never reaches our server. We hold the joining code for a few minutes, and nothing else.”

Q. Does it work without internet?

“Partly, and we are honest about it. The sign videos are saved on the phone after the first use, so they play with no network. Joining two devices needs a network. We would rather say that than promise offline and fail on a ward.”

Q. Who signs? Did deaf people help you build this?

“The clips are signed by deaf signers, and a reviewer approves every clip before it goes live. The app will not show a clip nobody approved.”

Q. How will you make money / how does a hospital pay for this?

“A hospital pays far more for one sign language helper than for this. Our plan is a small yearly fee per hospital, with the clip library shared across all of them. The cost of adding the tenth hospital is close to nothing.”

Q. What is not finished?

“Three things. The clip library is small. Two devices need a network to join. And it is not approved for clinical use — it is a pilot, and we say that on the screen. We would rather list those than pretend.”

Q. Can it do other Ghanaian languages?

“Text already goes to Twi. The sign videos are Ghanaian Sign Language, which is one language for the whole country, so that part does not change from region to region.”

Q. What did you build yourselves, and what did you use?

“We built the app, the sign matching, the safety rule, and the device-to-device link. We use an outside service for translating text to Twi. The sign clips are ours.”

Small words instead of big ones

If you catch yourself saying the word on the left, say the one on the right. Judges are tired by the tenth team. Simple words win.

DO NOT SAY	SAY
Peer-to-peer connection	The two devices talk straight to each other
Utilise, leverage	Use
Facilitate communication	Help them talk
Robust, seamless, cutting-edge	(say nothing — show it instead)

Deploy / deployment	Put it online / it is online
Our solution	Our app, or just “it”
Stakeholders	Doctors, nurses, patients
Scalable	It costs us almost nothing to add the next hospital
Validation / the gate	The app checks it, and refuses if a word is missing
Onboarding	Getting started

If something goes wrong on stage

- **A video is slow.** Keep talking. Say: “It is loading it for the first time. After this it is saved on the phone.” Then go on.
- **The two devices will not join.** Say: “Let me show you the one-screen way, which is what most wards will use anyway,” and switch to the shared screen. Do not try the code three times.
- **A screen freezes.** Reload it. The app is built to come back to the same visit. Say: “It comes back to where we were — a nurse should not lose a visit because a phone slept.”
- **You lose your place.** Go to the medicine screen. It is the strongest part. Finish there.
- **Time runs out.** Stop the demo and say the closing lines. Never run past your time. Judges take marks for it.

One last thing

Do not read the whole of these notes on stage. Read the green boxes once the night before, then twice in the morning. On stage, look up.

The app works. You only have to get out of its way.